



End of Life Care Procedure

1. Scope

Site	Operational Area	Applicable to
Armada Kalamunda Group	All services	Nursing, Midwifery, Medical and Allied Health

2. Purpose

This document describes the approach to managing End of Life Care (EOLC) throughout Armada Kalamunda Group (AKG).

This document is to be read in conjunction with the [EMHS End-of-Life and Palliative Care Strategy](#) and the [AKG Goals of Patient Care policy](#).

EOLC aims to:

- Deliver choices in maintaining quality of life of the dying person and their family, which also encompasses allowing a good death.
- Recognise and respond to the variability of patient needs and preferences including the diagnosis, religious / culture / emotional factors and care location
- Respond to the acute/unexpected deaths.

The dying patient has legal rights in relation to their medical treatment.

- The team delivering EOLC must:
 - Be multidisciplinary including both palliative and general health care where appropriate.
 - Provide continuity of care across service boundaries.
 - Minimise the interruption to the family's time with the patient.

3. End of life – definitions

End of life care (EOLC) in this document refers to the last few days of life when a person is irreversibly dying.

- This is also known as the terminal phase.

While there are situations in which it is possible to identify the terminal phase with some accuracy, many diseases have a natural history of progression and exacerbations which make the terminal phase difficult to identify.

Not all deaths are predictable or are the end result of chronic illness.

- They can be the result of trauma or an acute/unexpected clinical event.

Care at the end of life needs to be responsive and flexible to meet the varied scenarios that arise.

4. Procedure

4.1 Communicating with the family and next of kin

- Discussions between the multidisciplinary team (MDT) and the patient and / or family / Next of Kin (NOK) and carer should be continuous from the time the patient is identified as being at end of life to after death.

The communication should be:

- Open and honest.
- Prepared and where possible evidence based regarding the explanations provided.
- Responsive to the triggers for discussion (family and patient understanding).
- Focused on the care plan / pathway involving the patient / family / NOK and carer.

The topics covered are on a case by case decision, but key discussions are to be documented in the patient's health record. They may relate to:

- The diagnosis, prognosis, and options including the difficulty in predicting the duration.
- Ongoing assessment for needs of care.
- Patient and care giver's preferences including preferred place of death.
- The existence of Advanced Health Directives (AHDs), Goals of Care.
- Withdrawal of life support.
- Discussions with the MDT should be supplemented by printed information for the family, NOK and carer regarding the changes to expect and describe the role of the MDT and contact details of key people.

If disputes cannot be resolved regarding EOLC, parties can make an application to the [State Administrative Tribunal](#).

4.1.1 Advising of a death over the telephone

- When a patient dies in hospital, the family / NOK and carer are to be informed as soon as possible.
- Notification to the family / NOK and carer should be in person.
- Notification by telephone should only be done in exceptional circumstances.
 - If contact needs to be made with the family / NOK by telephone, the information given in the telephone call should be the truth about the death.
 - There should be no attempt to convey the impression that the person is still alive, or that death may be imminent, if in fact, the person has already died.
- Withholding news about the death or creating another impression about the patient's condition can only be done in exceptional circumstances with the reason recorded in the patient's health record or on an incident form.
- Allow the family / NOK time alone with the deceased.
- Refer to site specific (AHS and KH) policies, contacts and support services e.g. Pastoral Care.

4.2 Unexpected / acute death

- An unexpected death requires an immediate response to support the family and staff including planned communications between the family and the MDT, a referral to the social

worker and staff debriefing and support. The MDT may need to consider preferences included in an AHD and the family / NOK's and carer's request to be involved in the last offices.

- The AHD must be available to the MDT.

5. Care of the patient before death

5.1 Identifying the dying phase

The clinical deterioration of the patient needs to be monitored, responded to and continually re-assessed to identify those who are entering end of life.

- The decision to commence EOLC is made by a multidisciplinary team (MDT) and must be recorded in the patient's health record including:
 - The religious affiliation, cultural beliefs and spiritual practices / rituals relevant to EOLC.
 - The initial assessment: the patient status and care plan.
 - The diagnosis of the life limiting illness.
 - The reversible causes.
 - Physical and mental change along with the clinical deterioration.
 - The MDT, patient and family opinion.
 - The EOLC plan.
 - Clinical decisions e.g. medications, hydration and nutrition.
 - Changes to the Goals of Patient Care (GOPC) document AKMR 00G.
 - Ongoing assessment of the changing patient needs.
 - Communications within the MDT.
 - Communications between the MDT and the patient / family / next of kin (NOK) and carer.
 - Clinical handovers communicating Not for Cardiopulmonary Resuscitation (NFCPR) and other orders which are in place.

5.2 Delivering care

In achieving the aims of EOLC, it should be designed to:

- Respond to the psychological / spiritual need of the patient and family / NOK and carer.
- Anticipate and provide for distressing symptoms e.g. pain, fatigue, nausea, dyspnoea, confusion, restlessness, agitation.
- Address a range of needs e.g. mouth care, oxygen therapy and humidification, personal hygiene, nutrition and hydration, physical comfort and general observations.
- Include specialist advice to assist with the MDT decision-making e.g. for ceasing futile interventions or managing Internal Cardiac Defibrillator and artificial ventilation.
- Highlight additional referrals for the inpatient, including:
 - Social Worker.
 - Pastoral Care Service.
 - Oncology / Haematology Nurse Counsellor / Specialist Nurse Counsellor
 - Palliative Care Service.

- Aboriginal Liaison Officer.
- Complex Care Team.
- Allied Health
- Forget Me Not volunteer

Care may also focus on other patients in the case of a sudden death and for those who may be aware of imminent death, particularly if in shared or neighbouring rooms.

6. Advanced Health Directives and other orders

AHD forms and supporting information are available at the [Department of Health, Advance Care Planning](#) intranet site.

AHD documentation in the patient's health record must include:

- A clear record of each order e.g. on [the Goals of Patient Care \(GOPC\) document](#).
- Other specific treatment in the AHD e.g. that related to Medical Emergency Response.
- The name of the decision-maker/s, date and time of each order.
- Rationale for decisions and the names of other staff consulted about the decision.
- The extent of communication with the family / NOK and carer regarding any AHD and NFCPR Orders.
- The date by which the order is to be reviewed.
- Any other relevant information e.g. competency issues, role of a patient's guardian.
- If an order is rescinded, it is stamped "No Longer Applicable – Cancelled", with the Medical Officer's signature, name, date, time and removed from the prominent place in the patient's file.
- The rationale for ceasing the order must be documented in the health record and communicated to the Clinical Nurse Manager.

In order to meet the AHD policy requirements AKG needs to have a system in place for:

- Preparing and / or receiving AHDs in partnership with patients, families / NOK and carer.
- AHDs to include a wish for no resuscitation or:
 - A fully competent patient making an informed decision to refuse resuscitation.
 - A substitute decision-maker consenting to a NFCPR decision made by the MDT.
- A competent patient to make these decisions by:
 - Being adequately informed about the diagnosis and prognosis
 - Having his / her judgment not impaired or be in an alternated mental state, or is depressed
 - Having adequate time and counselling to permit the patient to adjust emotionally
- The MDTs which:
 - Make decisions in accordance with the AHDs when relevant and with the Enduring Power of Guardianship where one has been appointed.
 - Abide by AHDs recognising that this has the effect as if the decision was made by a patient with full legal capacity.
 - Deliver care consistent with the decisions regarding resuscitation and the extent to which a medical emergency response is exercised and are recorded in the patient's health record.

- Respond consistently with the [AKG Recognition and Response Acute Deterioration Policy](#)

6.1 Authority, periods of validity and communications including:

- GOPC form which are only signed with the approval of the doctor in charge of the patient's care or other Senior Medical Officer.
- Orders are valid for three months in the dialysis population and for patients admitted for terminal care, it is valid for the length of an admission.
- All other Orders should be reviewed within seven days or if there is a significant change in the patient's condition or diagnosis.
- Following each review, the Order is either discontinued or a further review date determined within seven days and recorded.
- All AHD and other Orders need to be prominently recorded in the patient's health record and be clearly communicated by the nurse on a shift-by-shift basis at handover.
- AHD's can be added as an alert on information systems.

7. Supporting information

The following documents support the implementation of this procedure:

- [Next of Kin Contact Procedure EMD-GUI-300-12](#)
- [Death of a Patient AKG-PRO-0388-14](#)
- [Goals of Patient Care AKG-POL-0181-18](#)
- [Advance Health Directive and Enduring Power of Guardianship AKG-GUI-0013-18](#)
- [Recognition and Response to Acute Deterioration AKG-POL-0388-14](#)

8. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 5 – Action 5.15 Comprehensive care at the end of life
- The health service organisation has processes to identify patients who are at the end of life that are consistent with the National Consensus Statement: Essential elements for safe and high-quality end-of-life care²²⁰
- Standard 5 – Action 5.16
- The health service organisation providing end-of-life care has processes to provide clinicians with access to specialist palliative care advice
- Standard 5 – Action 5.17
- The health service organisation has processes to ensure that current advance care plans:
 - a. Can be received from patients
 - b. Are documented in the patient's healthcare record
- Standard 5 – Action 5.18
- The health service organisation provides access to supervision and support for the workforce providing end-of-life care
- Standard 5 – Action 5.19

- The health service organisation has processes for routinely reviewing the safety and quality of end-of life care that is provided against the planned goals of care
- Standard 5 – Action 5.20
- Clinicians support patients, carers and families to make shared decisions about end-of-life care in accordance with the National Consensus Statement: Essential elements for safe and high-quality end-of-life care²²⁰

9. Policy owner (relating to these procedures)

Director Nursing and Midwifery

Enquiries relating to this procedure may be directed to:

Title: Director of Nursing and Midwifery
 Department: Hospital Wide
 Email: helen.fullarton@health.wa.gov.au

10. Review

This procedure will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
V3	03/01/2024	03/01/2027	Scheduled review

11. Authorisation

This procedure has been authorised and issued by the Director Nursing and Midwifery.

Authorised by	Helen Fullarton – Director Nursing and Midwifery
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